

Sample ID : **C21036**Patient Name : **KAVITA MANISH AGARWAL**Referred by Dr. : **SANJAY GALA**

Age/Sex : 48 / F

Sample On : 26-Mar-26 12:52:00 PM

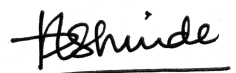
Reported On : 26-Mar-26 17:38

Past readings if any of mobile no :2223823613

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PRIYANKA P

Tests	Results	Biological Reference Interval
Fasting Blood Sugar Method:Hexokinase Past readings 09-Jan-26 209 13-Dec-25 192 15-Nov-25 125 11-Sep-25 126.0 28-May-25 166.0	<u>196.0</u>	70 -- 100 mg /dl
Post Lunch Blood Sugar Method:Hexokinase Past readings 09-Jan-26 313 15-Nov-25 219 28-May-25 179.0 28-Mar-25 193.0 14-Nov-24 157.0	<u>214</u>	70 -- 140 mg /dl
Notes : 1. Patients are advised to take diabetic medicines/Insulin on the day of testing as per the instructions of the treating physician. 2. Post lunch sugar can be lower than fasting sugar.		
Blood Urea Nitrogen Method:Urease, Serum Sample	<u>6.0</u>	8 -- 23 mg/dl
Serum Creatinine Method:Jaffe's,Traceable to IDMS	0.71	0.5 -- 1.2 mg/dl
Past readings 15-Nov-25 0.64 11-Sep-25 0.69 22-Jan-25 0.7 09-Jul-24 0.61 04-Dec-23 0.63		



Dr.Preeti Shinde
M.M.C. Reg.No.2003/05/2193
D.C.P. D.N.B. (Pathology)

28-Mar-26

5:07:30PM

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Dr.Madhukar Bhide's

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Tel No. 022 61381600 Email Id : info@bhidelab.net
Dr.Milind Bhide M.D. (Pathology)
Dr.Shilpa Bhide M.D. (Microbiology)
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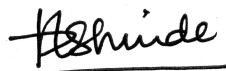
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PRIYANKA P

Tests

Results

Biological Reference Interval



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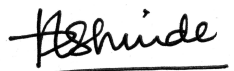
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PRIYANKA P

Complete Blood Count

Tests	Results	Biological Reference Interval
Erythrocyte Count Method:Sheath Flow DC Detection Past readings 15-Nov-25 5.09 30-Apr-25 4.67 14-Nov-24 4.84 09-Jul-24 4.46 27-Sep-23 4.86	<u>4.87</u>	3.8 -- 4.8 mil per c.u. mm.
Haemoglobin Method:SLS-Hemoglobin Past readings 15-Nov-25 12.6 30-Apr-25 11.5 14-Nov-24 11.8 09-Jul-24 10.9 27-Sep-23 12	12	12 -- 15 gms/dl
Leucocyte Count Method:Flow cytometry - Semiconductor Laser Past readings 15-Nov-25 10670 30-Apr-25 8290 14-Nov-24 8210 09-Jul-24 7690 27-Sep-23 9630	8660	4000 -- 10000 per cu. mm.
R.B.C. Indices P.C.V. Method:Sheath Flow DC Detection M.C.V. Method:Calculated M.C.H. Method:Calculated M.C.H.C. Method:Calculated	38.9 <u>79.9</u> <u>24.6</u> <u>30.8</u>	36 -- 46 percent 83 -- 99 femtolitres 27 -- 32 pico-grams 31.5 -- 34.5 percent



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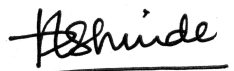
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Complete Blood Count

Tests	Results	Biological Reference Interval
R.D.W. - S.D. Method:Calculated	45.3	39 -- 46 fl.
<u>W.B.C. differential count</u>		
Neutrophils Method:Flow cytometry - Semiconductor Laser	68.1	40 -- 80 percent
Eosinophils Method:Flow cytometry - Semiconductor Laser	3.5	0 -- 6 percent
Lymphocytes Method:Flow cytometry - Semiconductor Laser	24.4	20 -- 40 percent
Basophils Method:Flow cytometry - Semiconductor Laser	0.2	0 -- 2 percent
Monocytes Method:Flow cytometry - Semiconductor Laser	3.8	2 -- 10 percent
<u>Peripheral Smear findings</u>		
WBC Method:Microscopy	Normal	
RBC Morphology Method:Microscopy	<u>Hypochromia</u>	
Platelet Count Method:Sheath Flow DC Detection	334000	150000 -- 400000 per cu. mm.
Past readings		
15-Nov-25	311000	
30-Apr-25	292000	
14-Nov-24	314000	
09-Jul-24	276000	
27-Sep-23	365000	

CBC done on fully automated analyzer XN1000/XN550

End of Report: Complete Blood Count


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GLYCOSYLATED HAEMOGLOBIN

Tests	Results	Biological Reference Interval
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Glycosylated Haemoglobin --- Hb A1c 10.27

Method:HPLC, EDTA Sample

Please see below

Past readings

15-Nov-25 8.07
11-Sep-25 8.38
30-Apr-25 8.24
22-Jan-25 7.69
14-Nov-24 8.52

Reference Interval :**Healthy Adults** : 4.0 to 5.7 %**Pre-Diabetes** : 5.7 - 6.5 %**Diabetes** : 6.5 % or more**Notes :**

All abnormal readings to be confirmed by repeat testing in patients with no prior history.

Treatment goals in Diabetic patients :

More than 8 % : Action suggested

Less than 7 % : Ideal level

Maintaining HbA1c levels to less than 7% will reduce risk of long term complications of Diabetes.

Diabetes Care 2018;41(Suppl. 1):S13-S27

Mean Plasma Glucose (mg/dl)**248****Notes :**

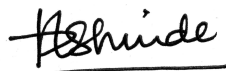
1. MPG is the average glucose level in the last 3 months.

2. Mean Plasma Glucose is estimated by using the formula
 $28.7 * (\% \text{ Glyco HB}) - 46.7$

3. Ideal level in diabetic patients is up to 154 mg/dl

4. MPG can be reduced in patients treated for anemia and in cases of decreased Red cell survival time like G6PD deficiency.

Ref : Clin Chemistry 54:11 1756 - 1758 (2008)

End of Report: GLYCOSYLATED HAEMOGLOBIN


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PRIYANKA P

T3, T4, TSH

Tests	Results	Biological Reference Interval
Total T3 (Tri-ido Thyronine) Method:ECLIA/CMIA Serum Sample Notes : 1. High values may be seen in Pregnancy due in increase in binding proteins. 2. Very low values may be seen with some medicines due to less conversion of T4 to T3	143	80- 200 ng/dl
Total T4 (Tetra ido- thyronine) Method:ECLIA/CLIA Serum Sample Notes : 1. High values may be seen in Pregnancy due to increase in binding proteins. 2. Low values may be seen in Euthyroid sick Syndrome	10.3	5.1 - 14.1 ug/dl
Ultra Sensitive T.S.H. Method:ECLIA/CLIA , Serum Sample Past readings 15-Nov-25 1.63 27-Sep-23 1.4745 Adult values : 0.38 - 5.0 : Normal > 10 : Suggestive of Hypothyroid status. 5-10 : Borderline. Repeat result after 2 weeks. < 0.2 : Low TSH value. Seen in hyperthyroid status, Non-Thyroidal illness, sometimes in normal patients. <0.01 : Low TSH value. Seen in hyperthyroid status, <0.001 : Very Low TSH value.Seen in hyperthyroidism. Elderly (>65 yrs) : Some studies suggest a higher upper limit. Newborn Values (Cord blood) : Upto 20 uiu /ml Pregnancy First Trimester : (0.02 - 2.81 uiu/ml) Clin Chem 57:6 913-926 (2011)	0.486	As mentioned below

End of Report: T3, T4, TSH

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Dr.Milind Bhide
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Thyroid Antibody

Tests	Results	Biological Reference Interval
Anti TPO antibody Method:CMIA, Serum Notes: 1. Anti-TPO antibodies are elevated in Hashimoto's Disease and Grave's disease. 2. Results should be used in conjunction with other test results and overall clinical picture. 3. Antibodies may be present in a low titre in normal individuals.	0.77	Upto 5.61 I.U./ml
Anti Thyroglubulin Antibody Method:CMIA, Serum Notes : 1. High levels of Anti thyroglobulin antibodies are seen along with Anti Microsomal antibodies in patients with Hashimoto's Thyroiditis,Primary Myxedema and Grave's disease 2. Up to 1 % of hypothyroid patients are positive for antithyroglobulin alone. 3. Positive tests are also seen in autoimmune diseases, diabetes and in elderly patients. 4. Positive results are also seen in a small percentage of healthy patients.	1.71	Up to 4.11 IU/ml
End of Report: Thyroid Antibody		

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Urine Routine

Tests	Results	Biological Reference Interval
<u>Physical Examination</u>		
Colour Method:Visual Observation	Yellow	
Reaction Method:pH Indicator	Acidic (6.0)	
Specific Gravity Method:Ion Exchange	1.015	
<u>Chemical Examination (Dip stick method)</u>		
Albumin Method:Acid Base Indicator	NIL	
Notes : Trace = approx. 10 mg% of albumin + = approx. 20 - 50 mg% of albumin ++ = approx. 50 - 250 mg% of albumin +++ = approx. 300 - 1500 mg% of albumin ++++ = approx. more than 2000 mg% of albumin		
Sugar Method:GOD/POD	2 %	
Acetone Method:Legal's	NIL	
Bile pigments Method:Diazonium Salt	NIL	
<u>Microscopic Examination</u>		
Red Blood Cells (per hpf.)	NIL	
Pus cells (per hpf.)	1-2	
Epithelial Cells	A few	
Casts	Nil	
Crystals	Nil	



Dr.Kunjal Karia
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Urine Routine

Tests	Results	Biological Reference Interval
Amorphous Material	Absent	

Method :

Urine Microscopy is done by Wet Mount

End of Report: Urine Routine



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